

1. Administrative & Study Identification

Full Study Title: A Phase I/II, Open-Label, Multi-Cohort Study to Evaluate the Efficacy and Safety of Cevostamab in Prior B Cell Maturation Antigen-Exposed Patients With Relapsed/Refractory Multiple Myeloma **Short Title/Study Title:** A Study Evaluating the Efficacy and Safety of Cevostamab in Prior B Cell Maturation Antigen (BCMA)-Exposed Participants With Relapsed/Refractory Multiple Myeloma (CAMMA 2) **Protocol Number:** C043476 **NCT Number:** NCT05535244 **Posting ID:** 936350482842535436 **Sponsor Name:** Hoffmann-La Roche (Roche) **Investigational Product:** Cevostamab (T-cell dependent bispecific antibody targeting FcRH5 and CD3) **Phase of Development:** Phase I/II (PHASE1, PHASE2) **Medical Monitor:** Hoffmann-La Roche Clinical Operations / Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy (Objective Response Rate [ORR] by investigator assessment) and safety (Percentage of Participants with Adverse Events) of cevostamab monotherapy via intravenous (IV) infusion in patients with relapsed/refractory multiple myeloma (RRMM). **Secondary Objectives:** To assess secondary efficacy measures (IRC ORR, DOR, CR rate), quality of life, pharmacokinetics, pharmacodynamics, and immunogenicity of the treatment.

2.2 Background & Rationale Multiple myeloma (MM) is an incurable bone marrow cancer. Patients with relapsed or refractory MM who have exhausted standard therapies, including those previously treated with B Cell Maturation Antigen (BCMA)-targeted therapies (such as CAR-T cells, bispecifics, or antibody-drug conjugates), have a high unmet medical need. Cevostamab binds to FcRH5 on myeloma cells and CD3 on T cells to activate the patient's immune system to destroy cancer cells. This study aims to provide a targeted immunotherapy option for this heavily pre-treated, triple-class refractory patient population.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm, Multi-Cohort **Blinding:** Open-label **Randomization:** N/A **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING)

3.2 Planned Enrollment & Duration Target **\$N\$:** 90 participants **Number of Sites:** Multicenter **Estimated Study Start:** 2022 **Estimated Primary Completion:** 26-Feb-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years with a documented diagnosis of MM based on standard International Myeloma Working Group (IMWG) criteria. 2. Evidence of progressive disease based on investigator's determination of response by IMWG criteria on or after their last dosing regimen. 3. Triple-class relapsed or refractory disease assigned to specific cohorts: - *Prior BCMA ADC or CAR-T Cohort*: Received a BCMA-targeted CAR-T or ADC therapy. - *Prior BCMA Bispecific Cohort*: Received a BCMA-targeting T-cell-dependent bispecific (TDB) antibody. 4. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 5. Life expectancy of at least 12 weeks. 6. Resolution of AEs from prior anti-cancer therapy to Grade ≤ 1 . 7. Agreement to protocol-specified assessments, including bone marrow biopsy and aspirate samples. 8. Agreement to remain abstinent or use protocol-specified contraception during the treatment period and for at least 5 months (females) or 2 months (males) post-treatment to avoid embryofetal exposure.

4.2 Exclusion Criteria 1. Inability to comply with protocol-mandated hospitalization. 2. Pregnancy, breastfeeding, or intention to become pregnant. 3. Prior treatment with cevostamab, another agent with the same target, or (for the prior ADC/CAR-T cohort) prior treatment with any TDB antibody. Prior CAR-T cell therapy within 12 weeks before first infusion. 4. Prior use of any monoclonal antibody (mAb), radioimmunoconjugate, or ADC within 4 weeks. Treatment with radiotherapy, chemotherapy, or other anti-cancer agent within 4 weeks or 5 half-lives. Prior treatment with systemic immunotherapeutic agents. 5. Administration of a live, attenuated vaccine within 4 weeks, or treatment with systemic immunosuppressive medications within 2 weeks prior to first study treatment. 6. Autologous stem cell transplantation (SCT) within 100 days; any prior allogeneic SCT or solid organ transplantation; major surgery within 4 weeks. 7. Circulating plasma cell count $>500/\mu\text{L}$ or 5% of peripheral blood white cells. 8. Medical history of autoimmune disease, amyloidosis, progressive multifocal leukoencephalopathy, significant cardiovascular disease (limiting CRS response), CNS disease, other malignancies within 2 years, severe allergic/anaphylactic reactions to mAb therapy, Grade ≥ 3 CRS or ICANS with prior bispecifics, HLH, MAS, or immune-mediated AEs from prior checkpoint inhibitors. Lesions in proximity to vital organs at risk for tumor flare decompensation. 9. Symptomatic active pulmonary disease or requiring supplemental oxygen. Known active bacterial, viral, fungal, mycobacterial, or parasitic infections requiring IV antimicrobials within 14 days. Active symptomatic COVID-19 infection. 10. Positive tests for EBV PCR (or chronic active EBV), CMV PCR, HBV, HCV, or HIV. 11. History of illicit drug or alcohol abuse within 12 months.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Cevostamab is administered in 21-day cycles. Cycle 1 utilizes step-up dosing (e.g., 0.3 mg on Day 1; 3.3 mg on Day 2, 3, or 4; and a target dose of 160 mg on Day 8). From Cycle 2 onwards, the target dose is given on Day 1 of each 21-day cycle. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Continued until disease progression or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Corticosteroids and tocilizumab (Actemra) for the management of protocol-specified adverse events like Cytokine Release Syndrome (CRS). **Prohibited:** Prior use of any monoclonal antibody, radioimmunoconjugate, ADC, or other systemic immunotherapeutic agents within 4 weeks of the first study treatment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, IMWG Disease Assessment, Bone Marrow Biopsy/Aspirate, Lab tests
Cycle 1	Days 1-21	In-hospital observation for at least 48 hours following the first 3 step-up doses; Safety & CRS monitoring
Subsequent Cycles	Day 1 (Every 21 Days)	Outpatient IV infusion, Efficacy Assessment, Safety and AE Checks
End of Study	30 Days Post-Last Dose	Final Safety Assessments, followed by survival/progression follow-up approx. every 3 months

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. **Objective Response Rate (ORR)** as Determined by the Investigator. 2. **Percentage of Participants with Adverse Events.**

7.2 Secondary Outcomes 1. **ORR as Determined by the Independent Review Committee (IRC).** 2. **Duration of Response (DOR).** 3. **Rate of Complete Response (CR) or Better.** (*Exploratory Endpoints include Quality of life assessments, Pharmacokinetics [PK], pharmacodynamics [PD], and immunogenicity*).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Intensive inpatient monitoring during Cycle 1 to assess for Cytokine Release Syndrome (CRS) and neurologic toxicities. **Serious Adverse Events (SAEs):** Reported continuously. Protocol-directed management of CRS involves the use of corticosteroids and tocilizumab (Actemra). **Data Monitoring**

Committee (DMC): Overseen by the sponsor to evaluate dose-limiting toxicities and continuous safety parameters.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) Population for efficacy outcomes; Safety Set for all participants who receive at least one dose of the investigational product. **Sample Size Justification:** Target $N=90$ is designed to evaluate preliminary efficacy (ORR) and characterize the safety profile across the different BCMA-exposed sub-cohorts. **Handling of Missing Data:** Standard imputation rules applied for QoL and efficacy endpoints as outlined in the core protocol.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Onsite clinical trial monitoring combined with centralized data reviews. **Audit Trail:** Standard electronic Case Report Form (eCRF) tracking with source data verification.

11. Study Identifiers & Governance

Primary ID: C043476 **Registry Number:** NCT05535244 **Posting ID:** 936350482842535436 **Sponsor/Lead Organization:** Hoffmann-La Roche (Roche) **Official Regulatory Title:** A Phase I/II, Open-Label, Multi-Cohort Study to Evaluate the Efficacy and Safety of Cevostamab in Prior B Cell Maturation Antigen-Exposed Patients With Relapsed/Refractory Multiple Myeloma

12. Clinical Framework (Multiple Myeloma Specific)

Primary Condition: Multiple Myeloma / Relapsed Refractory Multiple Myeloma (UMLS Preferred Name) **Diagnosis Threshold:** IMWG diagnostic criteria; Triple-class refractory status **Medical Methodology:** T-cell dependent bispecific antibody therapy (immunotherapy) **Optimization Target:** FcRH5 and CD3 dual-binding for T-cell activation and MM cell destruction **Ontology Codes:** - **MeSH Code:** D009101 - **HPO Code:** HP:0006775 - **SNOMED ID:** 109989006

13. Study Procedures & Methodology

Management Pathway: Step-up IV dosing designed to mitigate CRS **Education Protocol (HCP):** Clinical protocols for the prompt identification and treatment of CRS with corticosteroids and tocilizumab (Actemra; ATC Code: L04AC07) **Education Protocol**

(Patient): Counseling on the necessity of inpatient hospitalization for the first 3 doses, contraception requirements, and symptom awareness for immune-related adverse events **Quality Audit Mechanism:** Continuous IMWG response tracking and stringent AE/SAE electronic reporting

14. Observation & Follow-Up Schedule

Total Duration: Continued until disease progression or unacceptable toxicity, plus long-term follow-up **Onsite Visit Cadence:** Inpatient for Cycle 1; Day 1 of each subsequent 21-day cycle **Remote Monitoring Frequency:** Post-treatment survival and progression follow-up approximately every 3 months **Checklist Review Interval:** Every 21 days (prior to each new cycle administration)

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					